

إسم المريض ربيعاً : صفاء حسان محمد حسان
المحافظة : الجيزة الحي / المدينة : الجيزة
رقم البطاقة : 27604042100166 رقم التليفون : 01145651147
العمر(تاريخ الميلاد) : 04-04-1976 المهنة : بدون عمل
العنوان : المساكن الشعبية
تاريخ تسجيل الإستمارة : 29-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- /د ----- /د ----- /د.ا ----- /د.ا

ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: صفاء حسان محمد حسان National ID: 27604042100166	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

صفاء

حسان محمد حسان زيان

٦ ش محمود صالح

الجيزة - الجيزة

١٩٧٦/٠٤

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د. م. م.
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ربة منزل

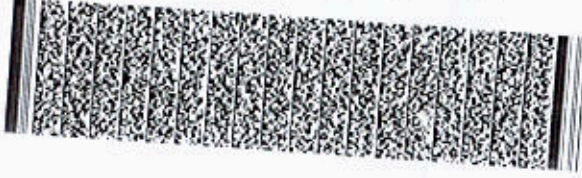
ارملة

معلمة

أنثى



البطاقة سارية حتى ٢٠٢٢/٠٤/٠٨



Name: صفاء حسان محمد		
MR. : 4233		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها The patient began cycle 2, day 1 of chemotherapy with gemzar 174 mg and carboplatin dosed to an AUC of 5; weight was recorded as 70 kg, height 156 cm, and BSA 1.74 m², and the same regimen was continued on cycle 2, day 15 with gemzar. On cycle 2, day 8 the patient received gemzar 1740 mg and carboplatin on day 1 as documented by the pharmacy. For cycle 3, day 1 the patient was administered gemzar 1740 mg together with carboplatin (AUC 5); pharmacy verification was noted for day 8. Cycle 4, day 1 continued with gemzar 1740 mg and carboplatin (AUC 5); pharmacy preparation was arranged for day 8, and laboratory testing on 30 March showed mildly elevated liver function tests, prompting the addition of sylimarine every 8 hours. Pharmacy confirmed receipt of gemzar 1740 mg on cycle 4, day 8, with laboratory results from 30 March accepted. Laboratory results were accepted for cycle 5, day 1, showing mildly elevated ALT and AST; pharmacy preparation was planned for day 8. On 20 May it was noted that the medication for cycle 5, day 1 had not been received; laboratory monitoring since 29 April was ongoing, and pharmacy preparation for day 8 was pending. For cycle 6, day 1 the patient received gemzar 1740 mg and carboplatin (AUC 5) with supportive treatment including ursoplus administered 12 hours after labs; a new CT chest-abdomen-pelvis was ordered, and pharmacy preparation was scheduled for day 8. The patient has a history of metastatic breast cancer to lung and lymph nodes since January 2020, previously treated with zoladex, ibrance, and aromasin. In December 2022, mediastinal lymph nodes and pulmonary nodules progressed, leading to initiation of xeloda in January 2025. Subsequent imaging in 2025 showed progression of pulmonary nodules and new lymphangitic carcinomatosis; multidisciplinary discussion recommended gemzar/carboplatin started on 30 March 2026, with radiologic response assessment after six full cycles showing stable disease of the pulmonary nodule (6 mm) and relative stability of the breast lesion. Two additional cycles were planned, and the patient proceeded to cycle 7, day 1 with gemzar/carboplatin, pharmacy preparation for day 8. CT comparison demonstrated a 6 mm pulmonary nodule stable disease and overall stable disease of the breast lesion; on cycle 8, day 1 the patient continued gemzar/carboplatin with mildly elevated liver enzymes (ALT 40 U/L, AST 42 U/L). Pharmacy preparation for day 8 included gemzar only.		
تحريراً في ,		
	الوقت :	19-09-2026 التاريخ :
هنا أسامه عبد الرحيم عبد البر		الطبيب المعالج :
يعتمد ,		

As of 18 August, the patient was awaiting comparative imaging; stable disease was noted on 27 June 2026, and three additional cycles were planned. New chemotherapy was scheduled for cycle 9, day 1 with gemzar 1740 mg and carboplatin (AUC 5), with day 8 pharmacy preparation and liver support measures.

For cycle 10, day 1 the patient received gemzar/carboplatin (AUC 5); laboratory results were accepted and pharmacy preparation was arranged for day 8. The patient has completed ten cycles of gemzar/carboplatin regimen to date.

MDT FORM -4 / 1 / 2028 G

<u>Name</u>	Umar Ali	<u>MRN</u>	4805/2022
		<u>ID</u>	01145651147
<u>Age</u>	49	<u>Phone</u>	97.881.17
<u>Menopausal status</u>	pre.meno	<u>Marital status</u>	
<u>NRS SCORE</u>		<u>Family history</u>	
<u>BMI</u>		<u>REASON OF MDT</u>	
<u>CO morbidity</u>			

Clinical Data

<u>BX:</u>		IDC		R	L		
<u>ER:</u>	8/8	<u>PR:</u>	5/8			<u>HER 2:</u>	-ve

Sonomammography:

1/2020 MBC to Lung & LNs :
Zoladex + Ibrance + Aromazine

Mets Workup:

12/2022 → newly developed med LN + nodes ^{TNM}

11/2025 → lung nodules 9mm (vs) 7mm + supraclavicular LN

Bx From LNs: Non necrotizing granulomatous

Recommendations:

lymphadenitis e Sarcoidosis.

→ Continue $x \in L_{\text{od}} a$.

11/2025 CT : PD of lymphangitis
pul nodules
chest LNs.

For many:

MDT OUTCOME & PLAN

for Review of sea in light

Dr. Bengar | Carbo

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Patient Name : الأستاذة / صفاء حسان محمد حسان	Request No. : 13001818
Patient No. : 27604042100166	Request Date : 12/02/2025
Path No. : 622/25	Verification Date : 05/03/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 48 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Known case of right breast cancer, on hormonal therapy with left enlarged supra-clavicular lymph nodes.
Nature of Specimen: Tru-cut biopsy, collected at 11/02/2025.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- Histopathological examination reveals cores of lymphoid tissue showing multiple granulomas formed of collections of histiocytes with multinucleated giant cells, No evidence of necrosis nor malignancy is detected in the examined sections.

Diagnosis:

- Left supra-clavicular lymph node, Tru-cut biopsy:
- Non necrotizing granulomatous lymphadenitis keeping with sarcoidosis.
 - Case for microbiological and serological correlation.

Signature

Prof. Dr. Amal Hareedy
Dr. Duaa Mohamed
Dr. Yasmine Essam

Verified by :

Printing Date : 05/03/2025 12:51:32

Page 1 of 1

Printing By : .marwa mohamed youss

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : صفاء حسان محمد حسان Pathology No : 14700/19
Sex : Age : 46 Hospital No. 23910/19
Clinical Data: Tru-cut biopsy from Rt.breast mass
Department : مريض عيادة خارجيه وحده جراحه - و Referring Dr. :

Electronically Authorized by: Mona Alabrak

Gross :

Four cores ranging from 0.2-1.5cm,totally submitted.

Histologic :

Examined material revealed an invasive breast carcinoma formed of irregular solid groups of malignant ductal epithelial cells with focal ductal differentiation 7/9. Anaplasia and mitosis are moderate. Surrounding stroma shows dense lymphocytic infiltrate and desmoplasia with evidence of lympho-vascular permeation.

No evidence of DCIS(ductal carcinoma insitu). ER, PR & HER2/neu will be done.

ER : 90% of tumor cells show strong staining (Allred score 8/8).

PR :positive in 10% of tumor cells show strong staining (Allred score 5/8).

Her2-neu : negative score +1

Ki67 index is 45%.

Diagnosis Breast,Right,Biopsy:Invasive duct carcinoma. ,Grade 2.

Category Malignant primary site (Single tumour)

Pathologists :

Dr. Mahitab Ibrahim Eltohamy

Dr.

Dr.

Yosra Nabil Faid

REPORT WAS SIGNED ON 17/11/2019

Verbal Revision Yosra Nabil Faid

Typist Sherean Sayed Ahmed Saad

Date of Issue : 18/12/2022

Patient Name: Safaa Hassaan Mohamed Hassaan Zayan	Date : 08/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 49 Year(s)
Medical ID: 4805/2022	Pat ID: 28151

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Patient with metastatic right breast cancer on hormone therapy, with newly developed bilateral supra-clavicular LNs, detected in CT for correlation

Stationary probably benign left breast nodule

Report:

- ACR b: scattered fibro-glandular parenchyma of both breasts.
- Right breast:
 - Right breast left partially indistinct residual subtle architectural distortion over an area about 60x65mm extending deep to the underlying muscle. Ultrasound shows parenchymal distortion at 40'clock with a para-sternal mass measured 1.2x1.8cm.
 - No left breast suspicious micro calcifications or architectural distortion detected.
 - By US no left breast solid or cystic masses
 - No pathologically enlarged axillary lymphadenopathy showing preserved fatty hilum, bilaterally.
 - Bilateral pathological supra-clavicular LNs, measured 0.9x0.9cm on the left side and 1.3cm with cortical thickness 3mm on the right side.

Opinion:

Newly developed pathological bilateral supra-clavicular LNs, for FNAC of the left side

Post hormone therapy status of right breast (BIRADS 6)

Signature Dr. Engy Abdallah

08/02/2025 5:06 PM

Not Approved

Patient Name: Safaa Hassaan Mohamed Hassaan Zayan	Date : 12/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 49 Year(s)
Medical ID: 4805/2022	Pat. ID: 28151

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:
Compared to previous study dated 29-04-2025.

Chest: -

- Adequately aerated both lungs
- **Progressive course** regarding size and number of the previously noted pulmonary nodules seen involving both lung lobes more on the right side, the largest still measures about 12 x 10 mm compared 0.9x0.8cm with newly developed reticulonodular infiltration suggesting lymphangitis carcinomatosa.
- **Progressive course** regarding right breast show: few nodules, the largest measures about 17 x 12 mm inseparable from underlying muscle compared to 1x0.8cm.
- **Regressive course** regarding the enlarged prevascular, pre and paratracheal and subcarinal lymph nodes with the largest at AP group measures 18 x 11 mm compared to previous study seen at subcarinal group measuring about 1.7 X 3.3 cm .
- Left lower lobe subsegmental consolidation patch.
- **Still noted:**
 - Left breast nodule is seen still measuring about 0.8 x 1 cm.
 - Bilateral basal atelectatic bands more at left lower lung lobe with mild overlying irregular pleural thickening.
 - Reticulation with atelectatic bands seen in lingular lobe.
 - Bilateral few supraclavicular lymph nodes, the largest is seen in left side measures about 17 x 10 mm compared to 1.7x1.2cm.
- No pleural or pericardial sac collection.
- Normal CT appearance of the heart and major thoracic vessels.

Abdomen & pelvis:

- The liver is of average size with diffuse homogenous low CT parenchymal attenuation and regular outline. No hepatic focal lesions or foci of abnormal enhancement. No intrahepatic biliary radicles dilatation.
- Patent homogeneously enhanced portal vein and its main branches
- Normal size & parenchymal texture of the spleen with no focal lesions. small splenule is noted .
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal CT appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still noted bulky uterus.
- No abdominal or pelvic masses or collections.
- Still noted of mesenteric fat plane smudging, likely panniculitis especially at midabdomen .
- No destructive lytic lesions.
- No vertebral structural collapse or wedging .
- No worriome lytic or sclerotic bony lesions.

Conclusion:

Stable response as detailed as compared to last study

DR/Reham khedr ,,, MD



Patient Name: Safaa Hassaan Mohamed Hassaan Zayan	Date : 29/04/2025
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 49 Year(s)
Medical ID: 4805/2022	Pat. ID: 28151

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Compared to previous study dated 01-01-2025.

I Chest: -

- Adequately aerated both lungs
- **Stationary course** regarding size and number of the previously noted pulmonary nodules seen involving both lung lobes more on the right side, the largest still measures about 0.9x0.8cm.
- **Still noted:**
 - Left breast nodule is seen measuring about 0.8 x 1 cm.
 - Right breast show: few nodules, the largest measures about 1x0.8cm.
 - Bilateral basal atelectatic bands more at left lower lung lobe with mild overlying irregular pleural thickening.
 - Reticulation with atelectatic bands seen in lingular lobe.
 - Enlarged prevascular, pre and paratracheal and subcarinal lymph nodes with the largest at subcarinal group still measuring about 1.7 X 3.3 cm cm.
 - Bilateral few supraclavicular lymph nodes, the largest is seen in left side measures about 1.7x1.2cm.
- No pulmonary masses, consolidation, cavitation or collapse.
- No pleural or pericardial sac collection.
- Normal CT appearance of the heart and major thoracic vessels.

II Abdomen & pelvis:

- The liver is of average size with diffuse homogenous low CT parenchymal attenuation and regular outline. No hepatic focal lesions or foci of abnormal enhancement. No intrahepatic biliary radicles dilatation.
- Patent homogeneously enhanced portal vein and its main branches
- Normal size & parenchymal texture of the spleen with no focal lesions.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal CT appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still noted bulky uterus.
- No abdominal or pelvic masses or collections.
- Still noted of mesenteric fat plane smudging, likely panniculitis.
- No destructive lytic lesions.

Opinion:

Overall stationary course since last study

Dr. Ahmed Elgazar

Dr. Mohamed Mohsen, M.D.



Patient Name: Safaa Hassaan Mohamed Hassaan Zayan	Date : 29/09/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 48 Year(s)
Medical ID: 4805/2022	Pat. ID: 28151

FOLLOWUP POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Compared to last study dated 16-3-2024.

I Chest: -

- Adequately aerated both lungs
- Newly developed solid nodule seen at right middle lobe measuring about 0.8 cm
- Stationary course of the previously noted pulmonary nodules seen involving both lung lobes more on the right side
- No pulmonary masses, consolidation, cavitation or collapse
- Left breast nodule is seen measuring about 0.8 x 1 cm.
- Bilateral basal atelectatic bands more at left lower lung lobe with mild overlying irregular pleural thickening
- Enlarged prevascular, pre and paratracheal and subcarinal lymph nodes with the largest at subcarinal group measuring about 2.1 x 2 cm
- No pleural or pericardial sac collection.
- Normal CT appearance of the heart and major thoracic vessels.

II Abdomen & pelvis:

- The liver is of average size with diffuse homogenous low CT parenchymal attenuation and regular outline. No hepatic focal lesions or foci of abnormal enhancement. No intrahepatic biliary radicles dilatation.
- Patent homogeneously enhanced portal vein and its main branches
- Normal size & parenchymal texture of the spleen with no focal lesions.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal CT appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.
- No destructive lytic lesions.

Opinion:

- Newly developed right middle lobe nodule with mild progressive course of the mediastinal lymphadenopathy
- Stationary course of the rest of the study

Dr. Ali Elsisy, MSc.

Dr. Ibrahim Abo El Magd, M.D.

HASSAN, SAFAA, Patient ID: 119023910 DOB: 04-Apr-1976 Date: 27-Nov-2022

Patient Lastname: HASSAN
Patient Firstname: SAFAA
HIS Patient ID: 119023910
RIS Patient ID: 240937
Date of Birth: 04/04/1976

Procedure: 72193, CT CHEST, ABDOMEN & PELVIS

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

EXAMINATION: CT Scan of the Chest, Abdomen and Pelvis With Contrast.

CLINICAL HISTORY: breast cancer

COMPARISON: 9/2020

FINDINGS:

1. Chest:

- " Stable right subfissural reticular density (fibrotic band).
- " No detectable sizable/ suspicious looking pulmonary nodules.
- " No mediastinal or hilar lymphadenopathy.
- " Normal cardiac size.
- " Unremarkable appearance of the thoracic aorta.
- " No pericardial or pleural effusion.

2. Abdomen and Pelvis:

- " No detectable hepatic focal lesions.
- " The spleen, the pancreas, both adrenal glands and both kidneys are within normal limits.
- " No lymphadenopathy.
- " No ascites.
- " The pelvic organs are within normal limits.
- " The visualized portions of the skeleton are unremarkable.

===== REPORT TEXT ENDE =====

HASSAN, SAFAA, Patient ID: 119023910 DOB: 04-Apr-1976 Date: 03-Dec-2022

Patient Lastname: HASSAN
Patient Firstname: SAFAA
HIS Patient ID: 119023910
RIS Patient ID: 240937
Date of Birth: 04/04/1976

Procedure: 93015, FDG PETCT
Medical Question:
Clinical Comment:

REPORT TEXT

F18-FDG PET/CT STUDY
(03 /12/2022)

CLINICAL HISTORY:

Female patient with metastatic right breast cancer on hormonal therapy.

TECHNIQUE:

Approximately 1 hour following intravenous injection of the radiopharmaceutical fluorodeoxyglucose (FDG), a total body PET-CT was acquired from base of skull to mid-thigh with the arms extended above the head. CT without contrast was acquired

Hepatic reference activity
NO IV contrast was administrated

Report NO Previous PET/CT study available for comparison , compared to the last report in 12-2019

Head and Neck:

- * No hypermetabolic FDG avid cervical nodal lesions detected.
- * Bilateral maxillary , ethmoidal and sphenoidal sinusitis.
- * Physiologic FDG uptake is seen in the oropharynx, salivary glands , thyroid gland and larynx.
- * The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- * NO sizable FDG avid breast lesion detected for sono-mammographic correlation.
- * Few prominent non FDG avid right axillary lymph nodes ; largest measures 10x5mm with no significant pathological FDG uptake for US correlation .
- * No hypermetabolic FDG avid left axillary nodal lesions detected.
- * Newly reported Multiple mildly enlarged FDG avid prevascular , retrocaval , subcarinal , aorto-pulmonary and bilateral hilar lymph nodes; largest measures 1.4x1cm with SUVmax 6.4
- * No FDG avid lung nodules or masses detected
- * There are no active pleural or pericardial abnormalities.
- * Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- * There is no hyper metabolic abdominal or pelvic lymphadenopathy.
- * The liver shows no FDG avid focal lesions.
- * Newly reported FDG avid left lumbar soft tissue density is seen measuring 1.6x1.1 cm with SUVmax 10.6 ? peritoneal lesion.
- * The spleen , pancreas show normal shape and normal FDG avidity.
- * There are no active adrenal nodules.
- * Physiologic FDG excretion is seen in the kidneys and bladder.

Musculoskeletal:

- * Normal FDG activity is seen in the axial skeleton. No focal active blastic or lytic lesions noted.